Carry

A wearable co-pilot for high-stakes professional conversations.

Carry listens through a wearable, privacy-filters every word, drafts the work product, remembers context, and prepares reviewable actions. Doctors are the first deep profession pack, not the limit of the platform.

Profession packs
Doctors first, lawyers next

Wearable-first
Ambient capture, no keyboard drag

Review-first
Drafts and actions stay accountable

THE USER

A doctor who spends the evening finishing notes from the morning.

DR

Dr. Rao, primary care physician

Sees 25 to 35 patients a day. Returning patients, new patients, overlapping histories.

1 During the visit

- Splits attention between the patient and the keyboard.
- Tries to recall what changed since the last visit.
- Holds allergies and medications in memory.

2 After the visit

- Writes the SOAP note from scattered memory.
- Sets coding, follow-ups, and referrals by hand.
- Documentation spills into personal time.

3 Across visits

- Prior context lives in long charts no one re-reads.
- A known allergy can be missed under time pressure.
- Continuity depends on one person remembering.

THE PROBLEM

Documentation steals time, and continuity is fragile.

Two costs sit on every clinician. The administrative load that follows each visit, and the risk that critical context from earlier visits does not surface at the moment it matters.

Time lost to administration

- Clinicians spend close to **two hours on paperwork for every hour** of patient care.
- Notes are reconstructed from memory, hours after the conversation.
- Coding, follow-ups, and referrals are manual and easy to defer.

Context that does not carry

- A **penicillin allergy** noted last visit may not surface before the next prescription.
- Prior medications and decisions are buried in long charts.
- Continuity depends on a human re-reading everything, every time.

THE JOURNEY

The same visit, with Carry listening.

BEFORE	The clinician opens the room with no briefing. They recall the last visit from memory.
PRE-READ	**Carry presents a one-screen briefing** built from earlier visits: known allergies, active medications, and open questions, carried forward automatically.
DURING	Carry listens, de-identifies on device, and **tracks clinical changes live**: symptoms, medication decisions, safety conflicts.
END	A **SOAP note, coding suggestions, and a follow-up plan** are drafted in seconds, each clearly marked for clinician review.
AFTER	The note syncs to the record, the follow-up is prepared, and **what was learned is carried into the next visit**.

HOW IT WORKS

Follow one sentence through the pipeline.

1 Capture

HEARD FROM PATIENT

"I am Anaya Mehta. Amoxicillin should help the throat."

2 Privacy filter

SENT FOR PROCESSING

"I am [PERSON_1] . Amoxicillin should help the throat."

3 Understand live

EXTRACTED

Medication **amoxicillin**

Penicillin allergy on file

4 Draft

DRAFTED FOR REVIEW

Switch to azithromycin

SOAP note

ICD-10 J02.9

5 Remember and act

AFTER SIGN-OFF

Synced to Notion

Follow-up booked

Allergy carried forward

NET RESULT

A risky prescription, caught and corrected before sign-off.

The note is drafted, the follow-up is ready, and the allergy is remembered for next time.

THE PRODUCT

Today: the pre-visit briefing

A returning patient. The briefing is built only from what Carry captured in earlier visits, with known allergies and medications carried forward.

Live dashboard

C Carry

Today
Command center

Live Visit
Active conversation

Patient
Longitudinal record

Timeline
Clinical history

Knowledge Graph
Relationships

TUESDAY CLINIC

Good morning, Dr. Rao

DEMO Visit 1 Visit 2 | Reset record | Begin next visit

AM **Anaya Mehta** — Returning patient
34, she/her · MRN-4821 · 09:30

Pre-visit briefing, built from what Carry captured in earlier visits.

CARRIED FORWARD BY CARRY
Penicillin allergy | Seasonal allergic rhinitis | Cetirizine 10 mg PO PRN

LAST VISIT
Seasonal allergic rhinitis. Started Cetirizine 10 mg PO PRN

CURRENT MEDICATIONS
Cetirizine 10 mg PO PRN

ALLERGIES
Penicillin

RECENT SYMPTOMS
Sneezing | Nasal congestion | Itchy eyes

UNRESOLVED
- Confirm symptom course since last visit

SUGGESTED AGENDA
1. Characterize current sore throat and fever
2. Confirm known allergies before any antibiotic
3. Assess need for rapid strep testing

Needs attention

1 **Notes captured** Visit drafts on record
1 **Allergies known** Carried into every visit
1 **Active medications** From captured visits
1 **Conditions tracked** Across the record

Recent updates

- Knowledge graph and timeline now span both visits
- Record updated from the latest visit
- Symptom noted: nasal congestion
- Allergy confirmed

THE PRODUCT

Live Visit: the safety catch

An antibiotic is proposed, the patient states a penicillin allergy, and Carry strikes the unsafe drug, flags the conflict, and tracks the safe alternative, as it happens.

Allergy conflict detected

C Carry

Today
Command center

Live Visit
Active conversation

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Timeline
Clinical history

Knowledge Graph
Relationships

LIVE VISIT · ANAYA MEHTA · VISIT 2

Conversation

Understanding

Visit in progress

Live transcript

10 exchanges

No cough, no chest pain, and breathing is okay.

Clinician

This could be bacterial pharyngitis. I was going to start amoxicillin 500 milligrams by mouth twice daily for ten days.

Patient

Wait, I am allergic to penicillin and amoxicillin. I get hives and swelling.

Clinician

Thanks for confirming. Do not take amoxicillin. We will avoid penicillin medicines.

Clinician

Instead, I will use azithromycin: 500 milligrams by mouth today, then 250 milligrams by mouth once daily for the next four days.

Patient

Clinical intelligence

Updated just now

LIVE PROCESSING

Understanding now

Last heard: 8s ago · Context: 0 / 4 turns

NOTE DRAFT

Drafting

- **S** Sore throat, fever, and swollen glands, 4 days
- **O** sore throat, fever, swollen glands
- **A** Working assessment in progress
- **P**

SYMPTOMS

- sore throat
- fever
- swollen glands

MEDICATIONS

THE PRODUCT

Final draft, ready for review

SOAP note, medication decision trail, coding suggestions, and a follow-up plan.
Every item is a draft that requires clinician sign-off before anything is final.

Clinician review required

C Carry

Today
Command center

Live Visit
Active conversation

Patient
Longitudinal record

Timeline
Clinical history

Knowledge Graph
Relationships

LIVE VISIT · ANAYA MEHTA · VISIT 2

Conversation

Visit complete

Begin next visit

Live transcript

10 exchanges

No cough, no chest pain, and breathing is okay.

Clinician

This could be bacterial pharyngitis. I was going to start amoxicillin 500 milligrams by mouth twice daily for ten days.

Patient

Wait, I am allergic to penicillin and amoxicillin. I get hives and swelling.

Clinician

Thanks for confirming. Do not take amoxicillin. We will avoid penicillin medicines.

Clinician

Instead, I will use azithromycin: 500 milligrams by mouth today, then 250 milligrams by mouth once daily for the next four days.

Patient

Clinical intelligence

Updated 5s ago

LIVE PROCESSING — Visit complete

- Last heard: 43s ago
- Context: 0 / 4 turns

NOTE DRAFT — Ready for review

S Sore throat, fever, and swollen glands The patient presents with a 4-day history of sore throat, fever, and swollen glands. Denies cough, chest pain, or shortness of breath. Allergic to penicillin and amoxicillin, which cause hives and swelling. Positive for fever. Positive for sore throat and swollen glands. Negative for cough and difficulty breathing. Negative for chest pain.

O Not recorded. Not performed/discussed in transcript.

A Suspected bacterial pharyngitis true

P Azithromycin 500 mg PO on day 1, followed by 250 mg PO daily for 4 days (total 5 days). Amoxicillin 500 mg PO BID for 10 days (cancelled due to penicillin allergy). Instructed to seek urgent care if experiencing trouble breathing, facial swelling, rash, or worsening fever. Follow up in two days if symptoms do not improve.

THE PRODUCT

Continuity: the patient knowledge graph

Every captured fact is wired to the visit where Carry learned it. The graph and timeline are built from real stored memory, not a fixed layout, and grow with each visit.

Built from live memory

C Carry

Today
Command center

Live Visit
Active conversation

Patient
Longitudinal record

Timeline
Clinical history

Knowledge Graph
Relationships

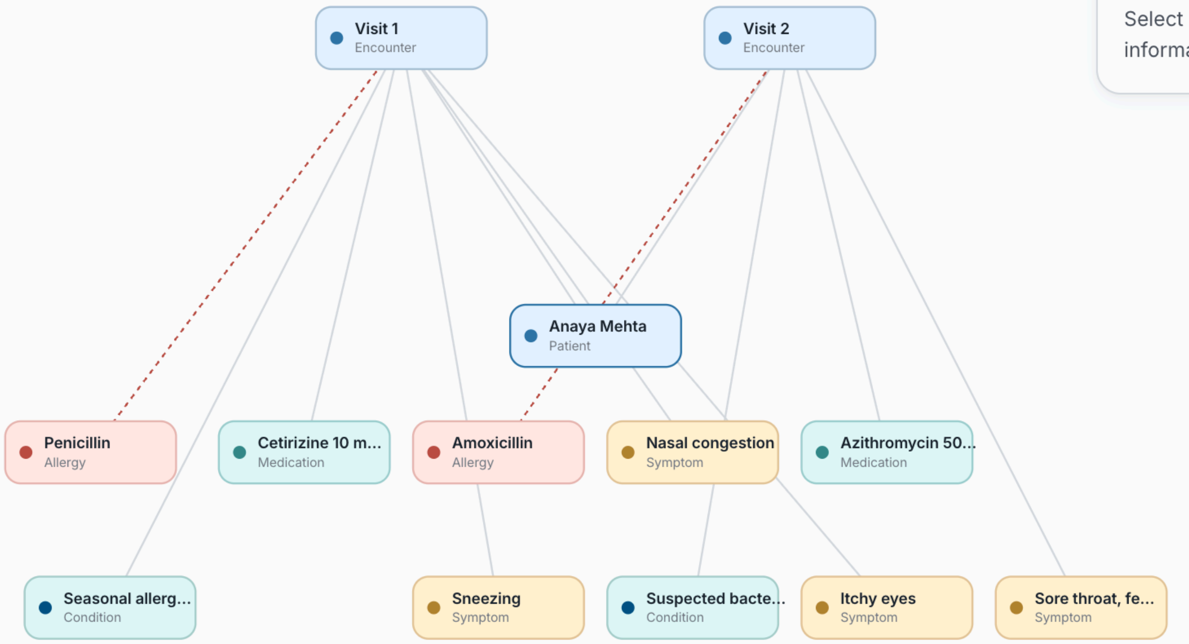
ANAYA MEHTA

Knowledge graph

Updated after the latest visit

NODE DETAIL

Select any node to trace where the information came from.



De-identified before anything leaves the device.

Captured audio
raw transcript

›

Privacy filter
on device

›

Processing
de-identified text

- Privacy filtering is the **first step** after capture, before any model sees the text.
- Direct identifiers are masked. **Clinical durations and relative dates are preserved** so the note stays accurate.
- Stored memory is filtered too: a blocklist keeps sensitive fields out of long-term context.
- Privacy-first, compliance-oriented architecture. The proof is visible in the transcript.

CAPTURED

My name is ~~Anaya Mehta~~. I have had a sore throat and fever for four days.

SENT FOR PROCESSING

My name is [PRIVATE_PERSON_1] . I have had a sore throat and fever for four days.

The dashboard shows both blocks side by side during the live visit, so the de-identification is auditable, not assumed.

TRUST ENGINE

Every AI action leaves a trace.

The risk with professional agents is not one bad answer. It is silent authority. Carry records what happened, what was allowed, what was drafted, and what still needs human review.

01 **Transcript event**
UTC timestamp, speaker, raw text, and source session.

02 **Privacy decision**
Captured text and sent-for-processing text, with redactions.

03 **Reasoning pass**
Incremental draft, final draft, model, prompt pack, and status.

04 **Memory write**
Only safe fields are carried forward, linked to the visit that taught them.

05 **Tool action**
Scoped integration call, result, and review state before it becomes work.

Scoped authority

Tools are enabled by named connections and config. No profession pack gets blanket access to external systems.

Scalekit · YAML policy

Human review gates

Doctor outputs are drafts. Carry can prepare the note and action trail, but sign-off stays with the professional.

Review required · No auto diagnosis

Replayable context

Chunks, redactions, outputs, memory updates, and sync results are stored so a bad run is investigable.

Session trace · SQLite memory

Containment

Live runs are session-bounded. Real transcript history is ignored before the UTC start cutoff.

Start cutoff · Explicit end

The engine is not a black box.

Every sentence, mask, reasoning pass, memory update, and tool action is tied back to the conversation that caused it.

ARCHITECTURE

Generic core, profession packs on top.

C Core platform

- Streaming transcript ingestion
- Privacy filter service
- Incremental and final passes
- Memory store and audit log

D Doctor pack

- SOAP note drafting
- Medication decision tracking
- Safety and allergy checks
- ICD-10 and follow-up plans

A Actions

- Record sync to Notion
- Calendar follow-ups
- Scoped, opt-in connections
- Every action is accountable

The next profession pack reuses the entire core. Only the outputs, prompts, and memory schema change.

CARRY

Carry the conversation into the work, and into the next visit.

SAVES TIME

The note, the coding, and the follow-up are drafted before the clinician leaves the room.

SAFER CONTINUITY

What was learned is carried forward, so a known allergy surfaces at the right moment.

DEPLOYS PRIVATELY

Privacy-first by construction, and able to run entirely inside the clinic.